

MODULE

3

Stomach & Spleen

The Anterior Circuit Continues – Head to Foot, Foot to Chest

Second channel pair of the 12 Primary Meridians – the two largest yet

This Module Covers:

- The Stomach Meridian of Foot-Yangming (ST) – 6 branches, 45 points
- The Spleen Meridian of Foot-Taiyin (SP) – 3 branches, 21 points
- How ST and SP close out the Anterior Circuit and hand off to Heart (HT)
- Ballroom & DanceSport relevance

Sourced from the Week 3 lecture deck (Dr. Zhang, 49 slides) and the Week 3 class transcript. Slides 1–13 of the deck are LU/LI review (already covered in Module 2) and are not repeated here except where the transcript adds something genuinely new.

How to Use This Module

ST and SP are the third and fourth of the 12 primary meridians, closing out the Anterior Circuit (LU to LI to ST to SP) before Qi hands off into the Posterior Circuit at the Heart Meridian. ST is by far the largest channel covered so far — 45 points across 6 branches — so this module leans harder on the branch-by-branch structure than Module 2 did.

1. The Stomach Meridian of Foot-Yangming (ST)

Meridian Clock: 7:00–9:00 a.m. Part of the Anterior Circuit, paired as Fu with the Spleen (Zang). Runs head to foot — the only Yang meridian of foot whose external course this module covers so far.

The Six Branches — Overview

Dr. Zhang's own slides organize this channel as six named branches rather than one continuous line. Learning the branch boundaries first makes the point-by-point course far easier to hold onto.

1. Facial branch (ascending): lateral side of the ala nasi to the forehead.
2. Facial branch (internal): a separate facial branch that pertains to the stomach and connects with the spleen internally.
3. The straight portion: supraclavicular fossa, through the nipple, down the abdomen to the groin.
4. Branch from the lower orifice of the stomach: descends inside the abdomen, down the thigh, to the lateral side of the tip of the 2nd toe.
5. The tibial branch: from 3 cun below the knee (ST36) to the lateral side of the middle toe.
6. The foot branch: from the dorsum of the foot to the medial tip of the great toe, linking with the Spleen Meridian.

Branch 1 — Facial, Ascending to the Forehead

Starts at the lateral side of the ala nasi (Yingxiang, LI20 — a crossing point, not an ST point itself). Ascends to the bridge of the nose, meeting the Bladder Meridian (Jingming, BL1). Turns downward along the lateral side of the nose (Chengqi, ST1), enters the upper gum, re-emerges, curves around the lips, and descends to meet the Conception Vessel at the mentolabial groove (Chengjiang, CV24). Runs posterolaterally across the lower cheek (Daying, ST5), winds along the mandible angle (Jiache, ST6), ascends in front of the ear through Xiaguan (ST7), then follows the anterior hairline (Touwei, ST8) to the forehead.

Branch 2 — Facial, Internal (Pertaining/Connecting)

From in front of Daying (ST5), descends past the anterior neck (Renying, ST9), the throat (ST10, ST11), enters the supraclavicular fossa (ST12), then internally pertains to the stomach, passes through the diaphragm, and connects with the spleen.

Branch 3 — The Straight Portion

From the supraclavicular fossa, descends through the nipple (Ruzhong, ST17) and by the umbilicus (ST19–ST29), entering the lateral side of the lower abdomen at Qichong (ST30).

The Week 3 transcript specifically calls out: “Only the stomach meridian passes through the nipple” — stated as a distinguishing landmark for telling ST apart from the Kidney, Spleen, and CV lines running nearby on the chest/abdomen. The transcript also discusses the relative spacing of these lines out from the midline (Conception Vessel), roughly CV to KI to ST to SP moving laterally — but the specific cun values given verbally were garbled in an auto-transcription (numbers like “four,” “six,” and “two” cun are audible but not clearly attributable to the correct lines). Flagged rather than stated as fact — will confirm exact spacing against a written source before using it for point-location purposes.

Branch 4 — From the Lower Orifice of the Stomach

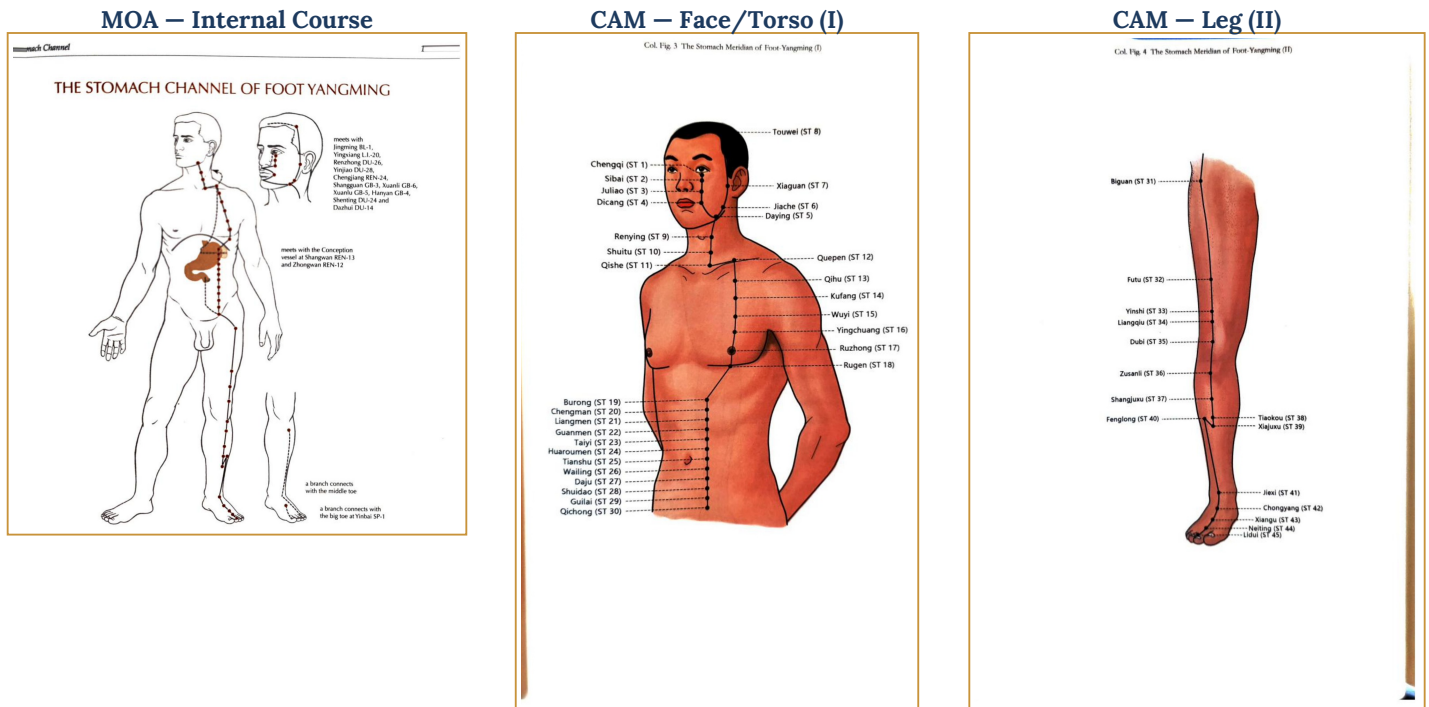
Descends inside the abdomen from the lower orifice of the stomach, rejoining the straight portion near Qichong (ST30), traverses the anterior thigh (Biguan, ST31), the knee via Futu (ST32), continues down the anterior-lateral tibia, crosses the dorsum of the foot, and ends at the lateral side of the tip of the 2nd toe (Lidui, ST45).

Branch 5 — The Tibial Branch

Separates 3 cun below the knee (Zusanli, ST36) and descends to the lateral side of the middle toe.

Branch 6 – The Foot Branch

Separates from the dorsum of the foot (Chongyang, ST42) and ends at the medial side of the tip of the great toe (Yinbai, SP1) – linking with the Spleen Meridian of Foot-Taiyin and handing off the Anterior Circuit's third leg.



Left: internal course and crossing points (MOA). Center/Right: the 45 external points, Chengqi (ST1) to Lidui (ST45), split across the source's two figures (CAM).

First and Last Points

	Point	Category
First point	Chengqi (ST1)	—
Last point	Lidui (ST45)	Jing-Well Point

Crossing Points (11 points)

Yingxiang (LI20), Jingming (BL1), Shangwan (GB3), Hanyan (GB4), Xuanli (GB6), Yinbai (SP1), Shenting (GV24), Shuigou (GV26), Zhongwan (CV12), Shangwan (CV13), Chengjiang (CV24). By far the most crossing points of any channel covered so far – consistent with ST being the largest and most widely-branching meridian.

Summary of the Stomach Meridian

- Pertaining organ: Stomach. Connecting organ: Spleen.
- Running course: head to foot. Starts lateral to the nose, terminates at the medial tip of the great toe (Yinbai, SP1), linking with the Spleen Meridian.
- Total points: 45 – the largest channel covered so far.

Main Syndromes & Indications

A. External course symptoms

- High fever, tidal fevers, flushed face.
- Sweating, sometimes chills, or pain in the eyes.
- Fever blisters (mouth and lips), sore throat.
- Swelling on the neck, facial paralysis, chest pain.
- Pain or distension along the channel in the leg and foot; coldness in the lower limb.

B. Internal organ (Stomach) symptoms

- Abdominal distension, fullness, edema.
- Persistent hunger.

Ballroom/Dance Relevance — the longest line on the body, and the one most loaded by extension (original synthesis, not lecture content)

ST runs the entire front of the body, head to toe — the single longest continuous line covered so far, and it's almost exactly the line placed under stretch in any deep backbend or extended contra-body line (American Smooth again, but also Rumba's hip-forward walk). 'Coldness in the lower limb' and leg/foot pain along the channel are worth knowing as a distinct pattern from the Posterior Circuit's own leg symptoms (Module 1) — anterior-leg complaints point here, posterior-leg complaints point to BL/KI instead.

2. The Spleen Meridian of Foot-Taiyin (SP)

Meridian Clock: 9:00–11:00 a.m. Part of the Anterior Circuit, paired as Zang with the Stomach (Fu). Closes out the Anterior Circuit before Qi flows into the Heart Meridian, opening the Posterior Circuit.

Running Course

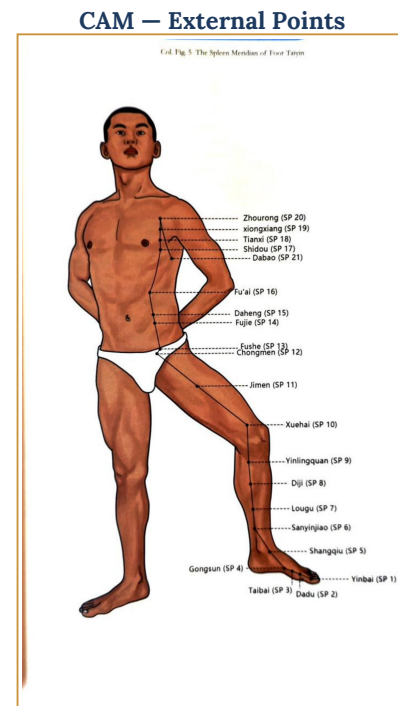
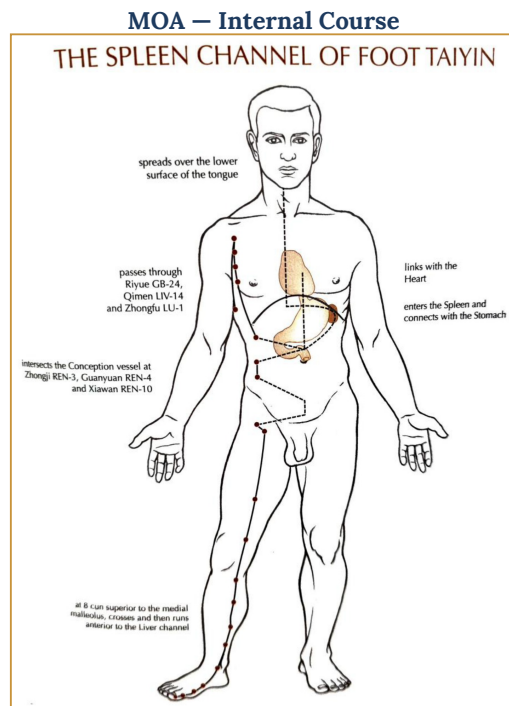
1. Starts at the tip of the big toe (Yinbai, SP1).
2. Runs the medial aspect of the foot at the junction of red and white skin, ascends in front of the medial malleolus (SP5).
3. Follows the medial aspect of the leg and the posterior aspect of the tibia.
4. Crosses in front of the Liver Meridian of Foot-Jueyin, 8 cun above the medial malleolus (SP9) — the same crossing named as the Liver Meridian's exception back in Module 1, Section 4.
5. Passes the anterior-medial knee and thigh, enters the abdomen, and pertains to the spleen, connecting with the stomach.

Branch 2 — Ascending from the Stomach

Passes through the diaphragm, runs alongside the esophagus, and spreads over the lower surface of the tongue, reaching the root of the tongue.

Branch 3 — Internal Connection

From the stomach, a separate branch goes through the diaphragm to the heart, linking with the Heart Meridian of Hand-Shaoyin — the first channel of the Posterior Circuit.



Left: internal course showing the Spleen/Stomach/Heart connections (MOA). Right: the 21 external points, Yinbai (SP1) to Dabao (SP21) (CAM).

First and Last Points

	Point	Category
First point	Yinbai (SP1)	Jing-Well Point
Last point	Dabao (SP21)	Major Luo-Connecting Point of the Spleen

Crossing Points (6 points)

Zhongji (CV3), Guanyuan (CV4), Xiawan (CV10), Riyue (GB24), Qimen (LR14), Zhongfu (LU1) — the last of these, Zhongfu, is the same point that opened the Lung Meridian in Module 2, now revisited as a crossing point rather than a starting point.

Summary of the Spleen Meridian

- Pertaining organ: Spleen. Connecting organ: Stomach.
- Running course: starts at the tip of the big toe, enters the abdomen, ascends through the diaphragm alongside the esophagus, reaches the tongue.
- The branch: stomach to diaphragm to heart, linking with the Heart Meridian of Hand-Shaoyin.
- Total points: 21.

Main Syndromes & Indications

A. External course symptoms

- Heaviness in the body or head.
- Fatigued limbs and emaciated muscles.
- Stiffness of the tongue, coldness along the medial side of the leg and knee.
- Edema in the foot or leg.

B. Internal organ (Spleen) symptoms

- Abdominal pain.
- Fullness or distension, diarrhea, poor digestion.
- Vomiting, hard lumps in the abdomen.
- Reduced appetite, constipation.

Ballroom/Dance Relevance – fatigue, heaviness, and the medial leg line (original synthesis, not lecture content)

SP's external symptom list – heaviness in the body, fatigued limbs, edema in the foot or leg – reads almost like a checklist for late-competition-day fatigue, especially in styles with sustained turnout (SP runs the medial leg, directly loaded by turned-out standing and deep plies in Latin/Rhythm). Tongue stiffness as a symptom is also a genuinely distinctive marker worth knowing – nothing else covered so far lists it.

3. Closing the Anterior Circuit

LU to LI to ST to SP is now complete – the full Anterior Circuit from Module 1, Section 6. SP's third branch (stomach to diaphragm to heart) is the actual handoff point into the Posterior Circuit: the next channel is the Heart Meridian of Hand-Shaoyin, Module 1's Section 6 table already names this sequence, and Module 4 will cover it directly.

Slide 48 of this week's deck states the underlying logic plainly: “The 12 main meridians are like the energy highways of the body, with each meridian having a specific active period and closely connected to its corresponding organ... mastering this pattern not only helps us understand the onset time of diseases but also provides important basis for precise treatment.” Four meridians and their clock windows are now covered: LU (3–5 a.m.), LI (5–7 a.m.), ST (7–9 a.m.), SP (9–11 a.m.) – exactly the first third of the 24-hour cycle.

4. Ballroom & DanceSport Relevance – Summary

Original synthesis, not lecture content – flagged as such throughout, consistent with Modules 1 and 2.

- ST and SP together cover the entire front-of-body and medial-leg lines – completing the Anterior Circuit's dance-relevant territory alongside LU/LI's arm and breath coverage from Module 2.
- Between the four Anterior Circuit channels, nearly every symptom list so far names fatigue, heaviness, or pain patterns that map onto real, specific training complaints rather than vague TCM abstraction – this is accumulating into real substance for the capstone's injury-pattern-to-channel framework.

5. Still Open / Not Yet Sourced

- The exact cun spacing of the abdominal/chest channels relative to the midline (CV, KI, ST, SP) – discussed verbally in the Week 3 transcript but the specific numbers were garbled in transcription. Will confirm against a written source before stating specific values.
- Meeting-point logic (Yuan-Source, Luo-Connecting, Five Shu, etc.) for ST and SP specifically – still deferred to the future Extraordinary Vessels / Special Points module, per Modules 1 and 2.

Source Notes

Primary source: Week_3.pdf (49 slides, Dr. Zhang, Week 3) and AC300Week3Transcription.txt (class transcript). Slides 1–13 and 38–47 of the deck are review of prior material (LU, LI) and quiz preparation; not repeated here. MOA and CAM figures are the same reference figures used across the weekly study kits. Section 4 is flagged separately as original synthesis.